

1. Administrative & Study Identification

Full Study Title: China Chronic Kidney Disease Registry Platform
Study Short Title/Acronym: C-INSIGHT Protocol Number: [Internal Study ID] NCT Number: NCT06117852 Sponsor Name: AstraZeneca
Investigational Product: N/A (Observational Registry) Phase of Development: N/A (Observational) Medical Monitor: [Name and Contact Information]

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To comprehensively gather data on the demographic and clinical characteristics, diagnosis and treatment patterns, clinical outcomes, and healthcare resource utilization among high-risk CKD patients in China to explore risk factors associated with CKD progression and clinical outcomes. Secondary Objectives: To support the development of predictive models for CKD progression and identify gaps between real-world management and diagnostic/treatment guidelines.

2.2 Background & Rationale Chronic Kidney Disease (CKD) poses a significant global health challenge due to its high prevalence, low awareness, poor prognosis, and substantial medical costs. High-risk factors such as severe proteinuria and uncontrolled hypertension further exacerbate the progression of CKD, leading to adverse kidney outcomes, cardiovascular diseases, hospitalizations, and increased mortality. Given these challenges, establishing a nationwide registry platform (C-INSIGHT) for high-risk CKD patients in China is essential to understand real-world disease profiles and enhance management strategies.

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational, Registry Control Type: N/A (Single-arm cohort) Blinding: Open-label / N/A Randomization: N/A

3.2 Planned Enrollment & Duration Target \$\$: Approximately 5,000 Chinese CKD patients Number of Sites: At least 37 study sites across China Estimated Study Start: Nov-2023 Estimated Primary Completion: Dec-2026 / 2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Patients at high risk of CKD progression. 2. Presence of severe proteinuria (urine albumin-to-creatinine ratio [$UACR$] ≥ 700 mg/g or urine protein-to-creatinine ratio [$UPCR$] ≥ 1000 mg/g). 3. OR presence of hypertension (systolic blood pressure [SBP] ≥ 130 mmHg).

4.2 Exclusion Criteria 1. Patients who have known severe hepatic injury (defined as Child-Pugh class C) based on medical history. 2. Patients undergoing renal replacement therapy (RRT) within 12 weeks prior to screening. 3. Patients on systemic immunosuppression prior to screening (except for stable maintenance therapy at least 3 months). 4. Patients with a life expectancy of less than 1 year, as judged by the investigator. 5. Patients who have participated in another interventional clinical trial and received an investigational drug within 1 month prior to enrollment.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: N/A (Observational Study mapping real-world standard of care) **Route of Administration:** N/A **Duration of Treatment:** Longitudinal follow-up from enrollment through study completion (2023 to 2027).

5.2 Concomitant & Prohibited Therapy Permitted: Routine clinical treatments and stable maintenance therapies (e.g., Prednisone ≤ 10 mg/day, Azathioprine ≤ 100 mg/day). **Prohibited:** Investigational drugs from other clinical trials within 1 month prior to enrollment.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
1	Day -14 to 0	Screening/Baseline: Informed Consent, Baseline demographic data, Clinical characteristics, Routine laboratory metrics ($UACR$, $eGFR$)
2	Annual	Interim: Annual Follow-Up Data Collection: Clinical outcomes, Diagnosis/treatment pattern updates, Healthcare utilization
3	Dec-2027	End of Study: Final Outcome Assessment, Exit records, All-cause mortality tracking

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Disease progression (progressive deterioration of renal function, including decline in estimated Glomerular Filtration Rate [$eGFR$] category and rapid progression). **Measurement Method:** Clinical evaluation and Electronic Health Records (EHR) review of laboratory parameters.

7.2 Secondary & Exploratory Endpoints 1. Clinical outcomes including renal outcomes, cardiovascular outcomes, cardio-kidney composite outcomes, and all-cause mortality. 2. Healthcare resource utilization. 3. Risk prediction models for CKD progression.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Routine capture of major adverse outcomes (e.g., adverse kidney events, hospitalizations) as part of registry tracking. **Serious Adverse Events (SAEs):** Captured within the framework of clinical endpoints (cardiovascular events, mortality). **Data Monitoring Committee (DMC):** N/A (Data governance methods are employed at selected centers to enhance data accuracy and efficiency).

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Enrolled High-Risk CKD Cohort (subdivided into CKD-HP [High Proteinuria] and CKD-HTN [Hypertension] subgroups). **Sample Size Justification:** $N=5,000$ to adequately capture diverse disease profiles, risk factors, and power the development of predictive models. **Handling of Missing Data:** Advanced data governance and electronic medical record extraction methodologies to minimize missing data.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Integration of traditional medical record review with electronic medical records (EMR) tracking. **Audit Trail:** Secure registry platform data collection and regular data governance audits to ensure screening and diagnostic accuracy.

11. Study Identifiers & Governance

Primary ID: C-INSIGHT **Registry Number:** NCT06117852 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** China Chronic Kidney Disease Registry Platform Study **Official Regulatory Title:** The China Chronic Kidney Disease Registry Platform Study (C-INSIGHT)

12. Clinical Framework (CKD Specific)

Primary Condition: Chronic Kidney Disease (CKD) in High-Risk Patients **Diagnosis Threshold:** $UACR \geq 700$ mg/g, $UPCR \geq 1000$ mg/g, or $SBP \geq 130$ mmHg **Medical Methodology:** Real-world

Evidence (RWE) / Guideline-Based Care Tracking **Optimization**
Target: Identification of gaps between real-world clinical practice and guideline-directed management

13. Study Procedures & Methodology

Management Pathway: Standard clinical pathway for high-risk CKD
Education Protocol (HCP): N/A (Observational) **Education Protocol (Patient):** N/A (Observational) **Quality Audit Mechanism:** Data governance protocols for EMR integration and clinical extraction accuracy

14. Observation & Follow-Up Schedule

Total Duration: Up to 4 years (Nov 2023 - Dec 2027) **Onsite Visit Cadence:** Annual clinical assessment mapping **Remote Monitoring Frequency:** N/A **Checklist Review Interval:** Annual registry database update

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				